

Bay Area Orthopedic Specialists, Inc.

Reza Tehrani, MD — Spine & Orthopedic Surgery

3010 Webster Street, Oakland, CA 94609

FOLLOW-UP NOTE

Patient Name:	David Park
Date of Birth:	September 24, 1986
Account Number:	BAOS-44712
Date of Visit:	August 14, 2024
Referring MD:	Marcus Hayes, MD (East Bay Family Medicine)

INTERVAL HISTORY

Mr. Park returns approximately 8 weeks after his initial consultation, having completed approximately 6 weeks of physical therapy at Active Recovery PT (10 of 12 prescribed sessions; he had to cancel two due to work travel). MRI from 07/01 was reviewed at the prior visit and demonstrated a right L4-L5 disc protrusion with L5 nerve root impingement.

The patient reports a mixed picture. His back pain at rest has improved meaningfully — he estimates 40-50% improvement in baseline pain. However, the radicular symptoms have not significantly improved, and he continues to experience right posterior thigh and lateral calf discomfort, particularly with prolonged sitting. He has resumed light walking but has not attempted to return to running. His PT (Hailey Chen) has communicated that he has plateaued on the radicular component despite full participation in directional preference exercises and neural mobilization.

He denies any progressive weakness, new numbness in a saddle distribution, or bowel/bladder dysfunction.

EXAMINATION

Gait remains non-antalgic. Lumbar ROM has improved — forward flexion is now to approximately 75 degrees without significant pain reproduction. Right SLR remains positive but at a higher angle (now positive at approximately 65 degrees, up from 50 degrees at the initial visit). Motor exam is unchanged: 5/5 strength throughout, with the previously noted very subtle weakness in right hip abduction and EHL no longer detectable on exam today, though I note this can be effort-dependent. Reflexes unchanged with mild right Achilles asymmetry. Sensation grossly intact.

ASSESSMENT

Right L5 radiculopathy 2/2 disc protrusion at L4-L5, with partial response to conservative management. Back pain has improved but radicular symptoms persist. Given that he has now had ~13 weeks of conservative care (6 weeks of PT, ongoing NSAIDs, activity modification) without resolution of radicular component, escalation to interventional pain management is reasonable.

PLAN

- 1. Referral to East Bay Pain Management for consideration of right L5 transforaminal epidural steroid injection. Discussed risks (bleeding, infection, dural puncture, transient pain flare) and expected benefits (symptom relief in 60-70% of patients with imaging-correlated radiculopathy).

2. Continue PT as currently structured for the remaining 2 sessions, then transition to home exercise program. Goal is maintenance of core stability and gradual return to running.
3. Continue NSAID coverage as tolerated.
4. Will plan to see him back 4-6 weeks after the injection to reassess. If radicular symptoms resolve, no further escalation needed. If symptoms persist or progress, surgical consultation would be next step — though I emphasized that surgical consideration would be reserved for failure of injection plus persistent neurologic findings or intolerable pain.

Total time spent: 25 minutes. The patient asked thoughtful questions about the surgical option and natural history of disc disease; I reassured him that most patients with this anatomy do not require surgery and that we have several intermediate options before that becomes a serious consideration.

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